

Primary Sexual Dysfunctions

- History and physical exam
 - Including sexual history
- No causes from comorbidities, stressors, relationships, or expectations
- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**

Arousal Disorders

Reduction or absence of at least 3:

- In a female or male, absent/reduced:
 - Sexual **interest/activity**
 - Erotic **thoughts/fantasies**
 - **Initiation/unreceptive** to sexual activity
 - Sexual **excitement or pleasure**
 - Response of arousal to **internal or external stimuli**
 - **Genital or non-genital sensations**
- **Almost all occasions** of sexual activity
- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**

Arousal Disorders

- Female Sexual Interest - Arousal Disorder
- Male Hypoactive Sexual Desire Disorder
- Differential Diagnosis (female or male)
 - Chronic medical condition
 - Substance use
 - Alcohol, marijuana, cocaine
 - Medications
 - Hypertension, psychotropic
 - Psychiatric/Psychological
 - Anxiety, Depression
 - (Peri-)Menopause
 - Radiation and chemotherapy
 - Lesion of the innervation of the genitalia
 - Unknown

Balon R, Segraves R (2019). Chapter 24. Sexual Dysfunction and Paraphilias. In Ebert M.H., Leckman J.F., Petrakis I.L. (Eds), *CURRENT Diagnosis & Treatment: Psychiatry*, 3e.

[Scholar Rx Brick: Sexual Dysfunctions](#)

Female Sexual Interest - Arousal Disorder

- Epidemiology
 - Prevalence – unknown
 - European study:
 - ~10-15% age < 50
 - ~25-35% age > 50
- Testing
 - Female Sexual Function Index
- Treatment
 - Treat any identified underlying cause
 - Lifestyle modifications
 - Bupropion
 - Topical lubricants/moisturizers
 - Estrogen
 - Testosterone
 - Cognitive Behavioral Therapy/Sex therapy

Male Hypoactive Sexual Desire Disorder

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- Epidemiology
 - Prevalence – 1.8% (ages 16-44 years)
- Testing
 - Psychological testing not required for diagnosis
- Treatment
 - Testosterone
 - Cognitive Behavioral Therapy (CBT)
- Prognosis
 - Transient – good
 - Chronic – poor

Female Orgasmic Disorder

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- **Epidemiology**
 - British sample – 3.7%
- **Differential Diagnosis**
 - Psychiatric
 - Anxiety, Depression
 - Drug induced
 - Antidepressants, sedatives
 - Chronic medical condition
 - Neurologic or endocrine disorders
 - Substance use
 - Alcohol, marijuana, cocaine
 - Relationship stressors
- **Clinical Evaluation**
 - Sexual history
 - Medication history
 - Physical exam
- **Testing**
 - Female Sexual Function Index
- **Treatment**
 - Change medication
 - Cognitive Behavioral Therapy/counseling
- **Prognosis good**
 - Long term committed relationships

Genito-Pelvic Pain / Penetration Disorder

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Recurrence of pain (at least one of following) during vaginal penetration or intercourse:

- Difficulty having intercourse
- Genito-/vulvovaginal or pelvic pain
- Fear or anxiety of genital pain or vaginal penetration
- Tensing or tightening of the pelvic floor muscles with attempted penetration

- **At least 6 months**
- **Distress**
- **Not** better fit for another **primary psychological condition**
- **Possible history of sexual/physical/mental abuse** resulting in increased response to perineal contact

- Dyspareunia → Genito-Pelvic Pain
- Vaginismus/vulvodynia → Penetration Disorder

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Genito-Pelvic Pain / Penetration Disorder

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- Epidemiology
 - North America – 15% c/o recurrent pain during intercourse
- Differential Diagnosis
 - Chronic medical illness
 - Endometriosis, constipation, irritable bowel syndrome, pelvic inflammatory disease, vaginal atrophy, lichen sclerosis or other perineal dermatologic disease
 - Psychiatric
 - Anxiety, post traumatic stress disorder, psychosomatic vs somatic disorder

Genito-Pelvic Pain / Penetration Disorder

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Genito-Pelvic Pain

- Treatment
 - Cognitive Behavioral Therapy
 - Vaginal hormonal creams/vaginal lubricants
 - Treat any underlying infection/etiology
 - Pelvic floor/physical therapy
- Prognosis- variable, most often poor

Penetration Disorder

- Treatment
 - Cognitive Behavioral Therapy
 - Benzodiazepine
 - Pelvic floor/physical therapy
 - OMM
- Prognosis – good with appropriate treatment

Male Erectile Disorder (ED)

- Epidemiology
 - < 40yo 1-9%
 - > 60yo 20-40%
- Differential Diagnosis
 - Age
 - Chronic medical condition
 - Hypertension, diabetes, other cardiovascular disease, hypogonadism
 - Psychological
 - Anxiety, depression, post-traumatic stress, prior poor experience
 - Medications
 - Antihypertensives, sedatives
 - Substance use
 - Smoking, alcohol, marijuana, cocaine
 - Surgery
 - Radiation therapy
- May lead to low self-esteem resulting in avoidance of sexual intercourse

Male Erectile Disorder (ED)

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- Clinical Evaluation
 - Associated embarrassment, self-doubt, and loss of self confidence
- Testing
 - International Inventory of Erectile Function
 - Labs
- Treatment
 - Modification of risk factors/lifestyle
 - Phosphodiesterase type 5 (PDE-5) inhibitors
 - Cognitive behavioral therapy

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Delayed Ejaculation

- Epidemiology
 - Exact prevalence is unknown
- Differential Diagnosis
 - Age
 - Medications
 - Antihypertensives, antidepressants/MAOI's/dopamine D-2 blockers, benzodiazepines/drugs causing alpha-adrenergic blockade)
 - Psychological
 - Depression
 - Chronic medical condition
 - Neurologic disorders, diabetes, hypertension, stroke, prostatitis
 - Substance useAlcohol, nicotine
 - Injury
 - Surgery

Delayed Ejaculation

- Testing
 - Pudendal nerve conduction study
- Treatment
 - Change medications (SSRI – side effect is delayed orgasm)
 - Education &/or psychosexual therapy
- Prognosis
 - Poor if psychological/medical in origin
 - Good if substance induced

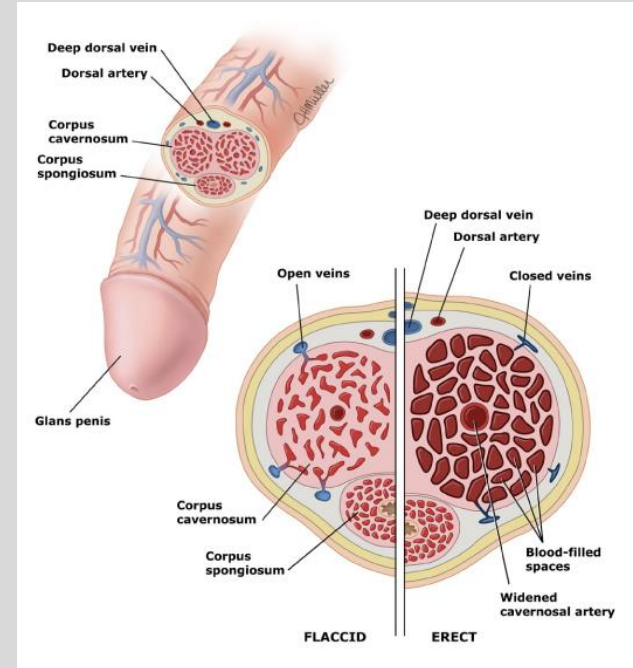
Premature (Early) Ejaculation

- Epidemiology
 - Suspect ~1-3%
 - Scandinavian study
- Differential diagnose same as other ejaculatory disorders
- Etiology
 - Minimal evidence
 - Psychological factors (anxiety, panic d/o, PTSD, relationship)
 - Unconscious anger
 - Learned in adolescent → performance anxiety
 - Genetic

Priapism

Persistent, usually painful erection lasting more than 4 hours in the absence of sexual excitation

- Epidemiology
 - 0.73 per 100,000 men per year
 - Bimodal peak distribution
 - 5 and 10 years in children
 - 20 to 50 years in adults
- Etiology
 - Pathologic engorgement of the corpora cavernosa but the glans penis and corpus spongiosum are not engorged and remain flaccid



Paraphilic Disorders

Paraphilia Behavior

- Preference for sexual behaviors (sexual fantasies or practices) outside of forms of courtship and genital stimulation considered normal within a given society

Paraphilic Disorder

- Paraphilic behavior with:
- Typically involve extreme, dangerous, and/or illegal sexual behaviors
- Feel personal distress about their interest or have a desire that involves another person's psychological distress, injury or death
- Rarely diagnosed
- NO FDA approved treatment for paraphilias
- DSM-5

Sexual Sadism Disorder

- Sexual gratification from the psychological and physical suffering, humiliation of another
 - GIVING abuse, humiliation, domination
- Real / 6 months / distress
- M>F
- Early adulthood
- Therapy (CBT) / medications (SSRI, GnRH agonist)

Pedophilic Disorder

- Have sexual desire for prepubescent children
 - (fantasies, urges or behaviors)
- 16 years old or 5 years older than victim for diagnosis
 - Victim usually younger than 13 years old
- 6 months / distress
- Puberty
- Fondling / oral sex
- Behavior is the disorder
- Highly treatment resistant (therapy/meds) / high recidivism

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Summary

Now, the student should be on the way to:

- ✓ 1. Define the general terms of sexual dysfunction and paraphilic disorders.
- ✓ 2. Distinguishing specific sexual dysfunctions.
- ✓ 3. Applying diagnostic criteria to identify sexual dysfunctions.
- ✓ 4. Developing a clinical management plan for sexual dysfunctions and paraphilic disorders.
- ✓ 5. Recognizing specific paraphilic disorders.

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